

Anti-GBM Antibody Mediated Glomerulonephritis

Agnes Fogo, MD

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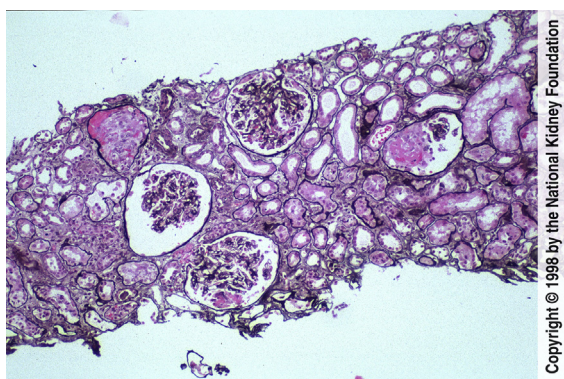


Fig 1. Focal segmental fibrinoid necrosis is evident, with remaining open glomeruli showing no proliferation or evidence of immune complexes in this case of anti-GBM antibody-mediated glomerulonephritis, a diagnosis confirmed by immunofluorescence studies (see Fig 3) (Jones silver stain; original magnification $\times 100$).

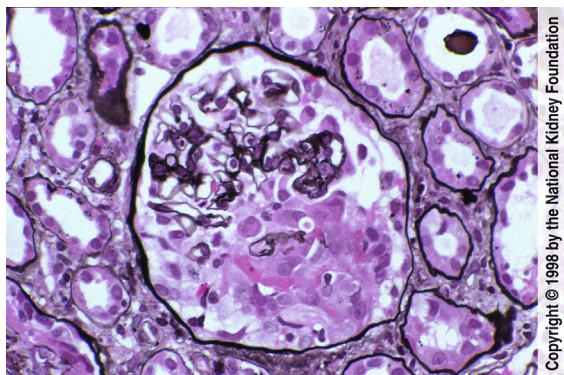


Fig 2. Early segmental fibrinoid necrosis with early cellular crescent formation without endocapillary proliferation or evidence of immune complexes in the remaining glomerular tuft. This light microscopic finding is that of a pauci-immune necrotizing glomerulonephritis, with a specific diagnosis of anti-GBM antibody-mediated glomerulonephritis made by immunofluorescence (see Fig 3) (Jones silver stain; original magnification $\times 400$).

From the Department of Pathology, Vanderbilt University Medical Center, Nashville, TN.

Medical Photographer: Brent Weedman; with assistance from Kim Soletz, MD, of the National Kidney Foundation's cyberNephrology™ Team.

Address author queries to Agnes Fogo, MD, Department of Pathology, Vanderbilt University Medical Center, MCN C-3310, Nashville, TN 37232. E-mail: Agnes.Fogo@vanderbilt.edu

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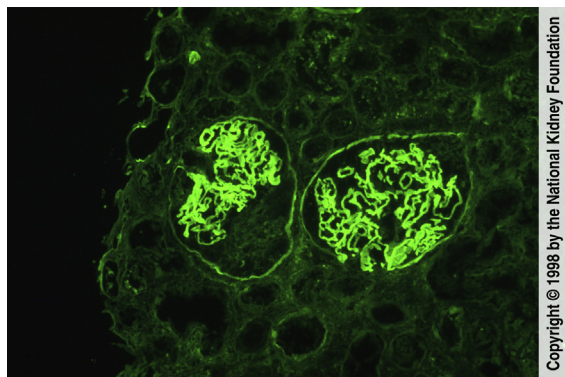


Fig 3. Smooth, linear staining of glomerular basement membranes with antibody to IgG and crescent formation in the glomerulus on the left, diagnostic findings of anti-GBM antibody-mediated glomerulonephritis (immunofluorescence with anti-IgG; original magnification $\times 200$).

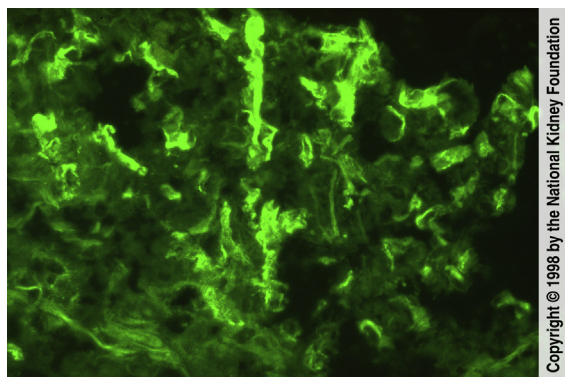


Fig 4. Lung with linear staining along alveolar walls with antibody for IgG in a patient with anti-GBM antibody-mediated glomerulonephritis and pulmonary hemorrhage, typical of Good-pasture's syndrome (immunofluorescence with anti-IgG; original magnification $\times 400$).